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01.00 Introduction

The Department of Health and Human Services Children's Personal Care Service (CPC) was developed in response to the special need for personal care services for children with special long term care needs.

It is a Title XIX (Medicaid) State Plan service available to Medicaid eligible individuals under the age of 21 who meet the Medical Necessity Criteria for the Children's Personal Care Service. **The Children's Personal Care Service is not a Medicaid waiver program.** The only service authorized under this program is Children's Personal Care.

CLTC Nurse Consultants are responsible for initial assessment and medical necessity determination for Children's Personal Care cases. Contract Care Coordinators are responsible for ongoing management and annual re-evaluation.

01.10 Policies and Procedures

01.11 Intake

A referral can be made by any knowledgeable source through CLTC Centralized Intake. Information about Medicaid applicants and recipients, including those persons referred to the CLTC Program, shall be safeguarded in accordance with the provisions of the South Carolina Department of Health and Human Services regulations governing confidentiality and HIPAA (Regulations 126-170, et seq.)

Confidential information includes, but is not limited to:

1. The name and address of the individual served and the service they receive;
2. Medical data, including diagnosis and past history of disease or disability;
3. Agency evaluation of information about a person;
4. The identity of persons or institutions that furnished health services to a person;
5. The social and economic conditions or circumstances of any person served; and,
6. Federal program information identified as confidential by appropriate federal authority.

All CLTC representatives/workers, contract workers, all persons receiving services from CLTC, and all other persons dealing with CLTC shall be informed of the Agency's policies and regulations with respect to safeguarding of confidential information.

Referral information is evaluated based on the Medical Necessity Criteria to determine if the applicant is appropriate for further screening. If not, potential resources which could meet the applicant's needs should be reviewed with the referral source or applicant.

In order for a referral to be considered appropriate for intake, the applicant must meet the following criteria:

1. Be under 21 years of age;
2. Be a resident of South Carolina (out-of-state applicants must intend to locate within the geographic area); and,
3. Be a current Medicaid recipient; if the applicant is not a current Medicaid

Recipient or has not applied for Medicaid, the applicant should be referred to the County Medicaid Eligibility (CME) office. If the applicant is a current Medicaid recipient and a "1" appears in the RSP indicator field, the RSP screen must be reviewed. Please refer to RSP overlap chart located in Phoenix HELP and note appropriate action to be taken.

4. Be seeking Children's Personal Care Service.
 - If the applicant is HIV positive, he/she must first be considered for the HIV/AIDS waiver. If determined inappropriate for the HIV/AIDS waiver, proceed with the Children's Personal Care Service evaluation.
 - A participant enrolled in the HIV or ID/RD home and community based waiver is not eligible for the Children's Personal Care Service because Personal Care II is a service available in the HIV or ID/RD waiver. Personal Care II services will be authorized through the respective waiver.
 - The Head and Spinal Cord Injury (HASCI) and Pervasive Developmental Disorder (PDD) waiver do not have Personal Care II (PC II) as a waiver service; therefore, if a participant is under 21 years of age and in the HASCI or PDD waiver, the participant may participate or continue to participate in the Children's Personal Care Service. The participant may receive attendant care through the HASCI waiver while receiving the Children's Personal Care Service.
 - Children enrolled in the Medicaid hospice benefit may receive Children's Personal Care Service; however, the Children's Personal Care Service is limited to 10 hours per week.
 - Children enrolled in a Medicaid HMO or PEP may receive Children's Personal Care Service.
 - Children participating in the Medically Complex Children Waiver (MCC) or Children's Private Duty Nursing Service (CPDN) will have Children's Personal Care Service authorized by MCC area.

If the applicant is appropriate for further screening, the Centralized Intake Program Assistant completes the appropriate screens in Phoenix. The case is then released to the appropriate Area Office and assigned to a CLTC Nurse Consultant.

01.12 Assessment Process

The **Nurse Consultant** schedules and completes an assessment visit with the applicant **within fifteen (15) business days** of case assignment. A home visit is required unless the parent or primary contact requests an alternate visit location. This exception must be approved by the CLTC Area Administrator or CLTC Lead Team Nurse Consultant. The visit must be made in the presence of a parent or knowledgeable other adult of the parent's choosing.

If the **Nurse Consultant** is unable to contact the parent or primary contact **within fifteen (15) business days**, a CLTC Notification Form 171 will be sent to the parent or primary contact requesting contact be made with CLTC in order to schedule a visit. If no response is received **within ten (10) calendar days**, the case may be terminated as "unable to locate."

Note: If an initial assessment for level of care for TEFRA or HASCI has been completed **within 180 days** of the Children's Personal Care application, the Children's Personal Care assessment can be completed by the **Nurse Consultant** by phone and a home visit is not required. An explanation of the service may be given via the phone. A consent form should be mailed to the parent or primary contact for signature. The CLTC **Nurse Consultant** may proceed with the assessment while awaiting the Consent Form but no further action should be taken prior to receipt of the form. The Children's Services Physician Information Form must be completed by the physician prior to medical necessity determination.

During the home visit, the **Nurse Consultant** should complete and obtain the following:

- Consent Form;
- Children's Services Rights and Responsibilities Statement;
- Assessment;
- Medications;
- Caregiver Supports/Permissions (only the caregiver name, phone number and tasks are required).

The following information is sent to the physician to determine medical necessity:

1. Children's Services Physician's Request Letter;
2. Copy of Consent Form; and
3. Children's Services Physician Information Form (The CLTC **Nurse Consultant** completes Section I).

If no response from the physician is received **within ten (10) business days**, a telephone call or letter should be initiated by the **Nurse Consultant** and documented in the Narrative. The parent or primary contact or referral source may be helpful in expediting receipt of information from the physician.

If the information received during the CLTC **Nurse Consultant's** assessment is conflicting or does not support the physician's recommendation of medical necessity, contact with other involved agencies may be indicated.

Once all information is complete, the case must be team staffed by two CLTC Nurse Consultants **within three (3) business days** to determine if the applicant meets the Medical Necessity Criteria. This team staffing will be documented in Phoenix. If the local CLTC office is unable to determine medical necessity, the case will be referred to the CLTC Central Office for consultation or determination. An email including the applicant's CLTC number will be sent to the Central Office Designee.

If the applicant is determined to meet the Medical Necessity Criteria, notification to the parent or primary contact will be made using the CLTC Notification Form 171.

If the applicant is determined to not meet the Medical Necessity Criteria, notification of denial must be made using the CLTC Notification Form 171 including the Appeals Notice.

01.13 Appeals

Any Medicaid applicant or recipient has the right to request an appeal of any decision by CLTC which adversely affects his/her eligibility status and/or receipt of services and/or assistance. This includes decisions to deny, reduce, suspend, or terminate a service. The formal process of review and adjudication of CLTC actions/determinations is done under the authority of Section 1-23-310 et seq., Code of Laws, State of South Carolina, 1976, as amended, and the South Carolina Department of Health and Human Services regulations Section 126-150, et seq.

The applicant, participant or designated representative must write a letter requesting an appeal within 30 days of the date of the official written notification issued by CLTC. The letter should be addressed to:

Appeals and Hearings Division
South Carolina Department of Health and Human Services
Post Office Box 8206
Columbia, South Carolina 29202-8206

Information regarding the individual's right to appeal and instructions for initiating an appeal are printed on the CLTC Notification Form 171.

01.14 Service Planning

The **Nurse Consultant** will initiate Service Planning for all applicants who meet the criteria for Children's Personal Care Service. The service plan must be

completed **within seven (7) business days** of the medical necessity determination.

In the interim, the CLTC **Nurse Consultant** is responsible for developing the Service Plan and obtaining prior approval for the Children's Personal Care hours.

Note: The service plan must be coordinated with the parent or primary contact. The Service Plan is accessible to the personal care provider in Phoenix and indicates the participant's specific needs to the personal care provider.

The following information will provide the CLTC **Nurse Consultant** with the basis for service planning:

1. The assessment completed by the CLTC Nurse Consultant;
2. Children's Services Physician Information Form;
3. Information gathered through home visit;

After reviewing all available information, the CLTC **Nurse Consultant** will determine:

1. The participant's needs;
2. The personal care activities needed; and,
2. The amount of time required for the Children's Personal Care Aide to meet these needs.

A copy of the completed service plan must be sent to the parent or primary contact.

The CLTC **Nurse Consultant** must request prior approval for the Children's Personal Care hours in the Phoenix Service Plan. The determination of the number of Children's Personal Care hours requested/ approved is based on the participant's needs and are not to exceed twenty-five (25) hours per week. Requests to exceed this limit must be prior approved by the Central Office

Note: If the participant is ventilator dependent, a referral to the Medically Complex Children's program for the Children's Private Duty Nursing (CPDN) program should be made for children under 18 years of age.

- The Children's Personal Care Aide is not to be left alone with a child requiring skilled care/ services. Any skilled care/ services will be completed by the parent, primary contact or other designated caregiver. Skilled care is not to be performed by the Children's Personal Care Aide.

- The Children's Personal Care Aide must be actively engaged in the completion of allowable tasks determined to be needed by the CLTC Nurse Consultant.
- The provision of this service does not include supervision of the child ("childcare") as a discrete task nor does it include down-time between tasks or time waiting for a task to be needed.
- Family members, the personal care provider, and the Children's Personal Care Aide should be counseled on the duties, responsibilities, and limitations of the service.

01.15 Program Enrollment

Following medical necessity determination, service planning and prior approval of the personal care hours, the application status is changed to "Participating." Upon this status change, Phoenix will automatically assign a Care Coordination provider. The **Nurse Consultant** will notify the Designated Support Staff to enter the Children's Personal Care enrollment date into Medicaid Management Information System (MMIS)/Recipient Special Program Summary (RSP) update screen. The RSP information is to be entered **within four (4) calendar days** of enrollment.

The CLTC **Nurse Consultant** will create and send the CLTC Notification Form 171 to the parent or primary contact and the primary case manager to confirm program entry.

01.16 Case Transfer for Ongoing Case Coordination

Once a Care Coordination provider has been assigned, the ongoing **Care Coordinator** will contact the assigned CLTC **Nurse Consultant within two (2) business days** to arrange a case transfer conference.

The CLTC **Nurse Consultant** will document the transfer conference in the participant's narrative record in Phoenix.

The newly assigned **Care Coordinator** will contact the parent or primary contact by phone to inform him/her of case transfer and to obtain personal care provider choices verbally. The Service Provider Choice Form may be mailed to obtain written provider choices. The parent or primary contact will make at least five (5) choices for Personal Care providers in order of preference. This contact and the personal care provider choices will be documented in the narrative.

01.17 Authorization

The **Care Coordinator** will create referrals to the chosen personal care providers

in the order of preference. Once a provider accepts the referral, the **Care Coordinator** will create and send the Service Provision Form to the personal care provider.

A family member can serve as the Children's Personal Care Aide for the child if he/she is NOT the

1. Parent (child under the age 18)
2. Foster parent
3. Step parent
4. Legally responsible guardian or
5. Health care power of attorney of the participant.

Family members must meet the same qualifications outlined in the Personal Care Services Scope of Service.

01.18 Monitoring

01.18.10 Monthly Monitoring

The participant must be Medicaid eligible in order to receive services. The **Care Coordinator** must verify Medicaid financial eligibility **monthly** through Phoenix or other means of Medicaid verification to ensure the participant has continued financial eligibility.

01.18.20 Quarterly Contact

The **Care Coordinator** is responsible for quarterly contacts with the parent or primary contact, provider and other agencies involved in the participant's care.

If it appears the participant no longer meets the Medical Necessity Criteria, a re-evaluation visit must be conducted.

The **Care Coordinator** must review the Care Call Activity and Exception Reports, narrate Care Call service monitoring, and follow-up on any problems noted. No more than 7 calendar days prior to contact, Care Call Client Activity Report and the Care Call A2 Exception Report, covering the time period since the last report, must be viewed.

This may be done by running the report prior to the contact or reviewing the report in real time.

The **Care Coordinator** should look for patterns that may indicate problems, not isolated cases.

For the Care Call monitoring, narrate review of the Care Call report from (date) to (date). Also any problems, subsequent interventions, and resolutions must be narrated.

Medicaid eligibility verification must be documented in the quarterly contact narrative in the participant's record.

The date the participant's status is changed to "Participating" establishes the due date of the initial quarterly contact.

The quarterly contact must be completed, documented and uploaded to the network by 11:59 PM on the last day of the month in which the contact is due.

If the Care Coordinator makes a referral to Child Protective Services (CPS) or if the child has an open CPS case, the **Care Coordinator** should contact the assigned Child Protective Services Worker quarterly and document this contact in the narrative. A report should be made in the Complaint Section of Phoenix when the referral is made or the open CPS case is discovered.

All quarterly contacts must be documented in the Narrative. The Narrative entry should, at a minimum, address verification of Medicaid eligibility, participant's condition, appropriateness of service needs/service plan, and delivery of services.

If in attempting to complete a quarterly contact, the **Care Coordinator** is unable to reach the parent or primary contact, a Notification Letter will be sent to the parent or primary contact requesting he/she contact the Care Coordinator to complete the required contact. If no response is received **within ten (10) calendar days** of mailing the Notification Letter, the Children's service application may be terminated.

01.18.30 Re-evaluation

At a minimum, the **Care Coordinator** must complete a formal re-evaluation visit to the participant's home by the last day of the month in which the re-evaluation is due.

The case must be team staffed with a CLTC **Nurse Consultant** to complete the medical necessity determination. The CLTC Area Office will receive a Phoenix Notification for team staffing. The Area Administrator or Lead Team will delegate team staffing to the CLTC Area Office Nurse Consultants or Lead Team Nurse Consultant.

The **Care Coordinator** must use Care Call to check in and out of the home for the visit.

The **Care Coordinator** must review the Care Call Activity and Exception Reports, narrate Care Call service monitoring, and follow-up on any problems noted. No more than 7 calendar days prior to contact, Care Call Client Activity Report and the Care Call A2 Exception Report, covering the time period since the last report, must be viewed. This may be done by running the report prior to the contact or reviewing the report in real time. The Care Coordinator should look for patterns that may indicate problems, not isolated cases. For the Care Call monitoring, narrate review of the Care Call report from (date) to (date).

Any problems, subsequent interventions, and resolutions must be narrated in the re-evaluation narrative.

Medicaid eligibility verification must be documented in the narrative.

The date of the medical necessity determination is the date from which the re-evaluation is scheduled.

All re-evaluation activities must be completed, documented and uploaded to the network by 11:59 PM on the last day of the month in which the re-evaluation is due.

For re-evaluation, the **Care Coordinator** should complete, obtain and review the following:

- Review consent form
- Review Children's Services Rights and Responsibilities Form
- Complete a new Assessment
- Update Medications
- Review Caregiver Supports/Permission and update if needed
- Obtain Children's Services Physician Information Form
- Team staff with a CLTC Nurse Consultant to determine if medically necessity criteria is met
- Develop a new service plan

The Physician's Information Form will be sent **prior to the assessment visit** in order to allow timely for return of the form. If no response from the physician is received **within ten (10) business days**, a telephone call or letter should be initiated by the **Care Coordinator** and documented in the Narrative. The parent or primary contact or referral source may be helpful in expediting receipt of information from the physician.

If the information received from the **Care Coordinator's** assessment is conflicting or does not support the physician's recommendation of medical necessity, contact with other involved agencies may be indicated.

If during the monitoring process a change occurs in the participant's service needs

or living arrangements, a review of the appropriateness of the service plan and a review of the number of hours authorized must be conducted. This review should be conducted by phone with the parent or primary contact. It may also include contact with the primary case manager or other involved agency.

A revision of the service plan and change in the number of hours authorized may be indicated. The **Care Coordinator** will complete a prior approval request for increased personal care hours in the service plan. The CLTC Area Office will receive a Phoenix Notification to review of the prior approval request. The Area Administrator will delegate this task to the CLTC Area Office Nurse Consultants and/ or Area Office Lead Team Nurse Consultant.

01.19 Medical Ineligibility

When a participant has been determined to not meet the medical necessity criteria, a Notification Letter is used to formally notify the parent or primary contact of this decision.

The case should be reviewed with the designated CLTC Area Office Lead Team or Area Administrator prior to rendering a final decision. If the Area Administrator determines the case requires further review, it should then be referred to the Central Office Designee for a final level of care decision. Once the level of care is determined, Central Office will notify the Care Coordinator of the Medical Necessity Determination and the required third signature may be signed by the Area Administrator, Lead Team or Central Office Designee.

If the applicant's final level of care determination is medically ineligible, the team completes and signs the Assessment level of care section in Phoenix. A Notification Letter including the Appeals Notice will be sent to the parent or primary contact. The Care Coordinator terminates the program application in Phoenix and if appropriate terminates the case.

01.20 Financial Ineligibility

If at any point a participant becomes financially ineligible, all services must be terminated and the case must be closed effective the date of financial ineligibility.

If **Care Coordinator** receives notification that a participant is losing financial eligibility, he/she must complete the following tasks:

- Document verification of financial ineligibility via Phoenix, letter from Social Security Administration, or written notice from Medicaid Eligibility.
- Notify involved agencies by telephone call of participant's termination from the Children's Personal Care program.

- Contact participant/primary contact to discuss termination, possible alternatives, and referral to other agencies.
- Complete a CLTC Notification Form 171 to notify participant/primary contact indicating the date of termination will be the date of financial ineligibility.
- Document all activities in the Narrative.
- Terminate Care Coordination and Children's Personal Care in Phoenix effective the same date as financial ineligibility.
- Complete Status Change section of Phoenix Applications to terminate the Children's Personal Care application and the case as "Financially Ineligible."

01.21 Transfer

01.21.10 Transfer to a Home and Community Based Waiver

If a participant enrolled in Children's Personal Care is to be admitted to a waiver (CLTC or DDSN), activities must be coordinated with the appropriate agency to ensure continuity of care. Adequate coordination allows the participant to transfer from the Children's Personal Care Program to a waiver without a break in services. Appropriate forms must be completed and forwarded as necessary.

If a participant between ages 18-21 requests or needs other waiver services, he/she may be considered for transfer to the Community Choices waiver. However, financial and medical eligibility for the waiver must be established before a transfer can occur.

If a transfer to a CLTC waiver is appropriate, the following policy and procedure must be followed:

1. The **Care Coordinator** must make an electronic referral for the waiver via the web address https://phoenix.scdhhs.gov/cltc_referrals/new.
2. The **Care Coordinator** must coordinate the transition with the receiving **Case Manager II** in order to avoid a break in Personal Care services.
3. The Children's Personal Care application must be closed in Phoenix by the **Care Coordinator**.
4. The **Care Coordinator** will notify the CLTC area office designee to update the MMIS RSP. The end date for the CHPC (Children's Personal Care) RSP indicator will be the day before enrollment in the waiver.

5. The receiving **Case Manager II** will issue a new service provision form to reflect the Waiver Personal Care II procedure code.

01.21.20 Transfer from a Home and Community Based Waiver

If a participant enrolled in a waiver (MCC or DDSN) chooses to enroll in Children's Personal Care, activities must be coordinated with the appropriate agency to ensure continuity of care. When a participant transfers from the Children's Personal Care Program to a waiver without a break in services, the waiting list does not apply. Appropriate forms must be completed and forwarded as necessary.

If a transfer from a CLTC waiver is appropriate, the **Waiver Case Manager** must make a Children's Personal Care referral electronically via the web address: https://phoenix.scdhhs.gov/cltc_referrals/new.

01.21.30 Transfer to another Care Coordination Provider

Upon receipt of notification of the participant's relocation outside of the Care Coordinator's service area, the **transferring Care Coordinator** will complete the following activities:

- Sends an email notification to the Central Office Designee.
- Enters the transfer information in the Narrative, indicating the new address, directions to home, telephone number, and a statement of the personal care provider's ability to continue service.
- Completes the CLTC Notification Form 171 to notify involved agencies, participant, primary contact, and/or knowledgeable other of the transfer.

If the authorized personal care provider indicates the service can continue in the new location, the Children's personal care authorization may remain open.

The **receiving Care Coordinator** completes the following activities:

- Reviews participant's information in Phoenix.
- Contacts the participant, primary contact, and/or knowledgeable other to update medical information and service needs.
- Verifies Medicaid eligibility.

- Reviews the open personal care authorization for appropriateness.
- Confirms with the personal care provider that the services may continue with the current authorization.
- Maintains the same schedule for quarterly monitoring and re-evaluation visits.

RSP information does not need to be re-entered.

01.22 Termination

If the participant's condition changes and the participant no longer meets the Medical Necessity Criteria,

- Care coordination and personal care services will be terminated.
- A written ten (10) calendar day notice must be given prior to terminating the personal care service and the case.
- The notification of this determination through the CLTC Notification Form 171, including the Appeals Notice.
- The date of and reason for closure must be indicated on the form (10 calendar days from the date of notification.)
- A copy of the CLTC Notification Form 171 must be sent to the parent or primary contact, personal care provider and the primary case manager at the same time.

Once the ten (10) day notice has expired, the **Care Coordinator** notifies the **Area Office Designee** to enter the termination date of the CHPC RSP into Medicaid Management Information System (MMIS) /Recipient Special Program Summary (RSP) update screen.

The RSP information is to be entered into MMIS **within four (4)** calendar days of closure.

Since the participant is only eligible for the Children's Personal Care through the end of the month the participant turns 21 years old, the ten (10) day notice must be given timely to ensure the case will be closed at the end of that month.

01.23 Case Filing and Documentation

Documentation is required to record any case activity in accordance to policy and

procedures. Appropriate documentation is mandatory. All pertinent actions related to CLTC's processing of the case and the Care Coordinator's service monitoring should be documented in the Narrative.

- Documentation should be related to the participant and should reflect factual information recorded in a professional manner.
- Only approved acceptable abbreviations are to be used.
- Correct spelling and proper grammar must be used.
- Extraneous comments unrelated to the participant's situation are not appropriate for inclusion in the case record.
- Personal opinions and judgmental statements are not appropriate for the participant record.
- Emails are not appropriate for the participant record.
- Quality assurance reviews/forms must not be filed in the participant record.

These standards for documentation apply to all correspondence and to **ALL** documentation in the Phoenix system. Narrative checklists and conversations are part of the narrative record.

If participant information is documented somewhere in the participant record, it does not have to be repeated.

When making a complaint in Phoenix, it **MAY** contain the CLTC number, provider name, provider worker name, and worker ID. It **MAY NOT** contain the participant name, participant Social Security or Medicaid number, or the names of relatives.

The Conversation Tab feature may be used rather than contacting the provider by telephone. Examples of appropriate uses of Conversation are informing the provider of a schedule change, hospitalization, etc. However, when informing the provider of information that requires immediate attention such as an immediate termination, a telephone call for verbal communication is necessary. All documentation guidelines apply to the Conversation Tab feature in Phoenix. Correct spelling and proper grammar must be utilized. Only approved abbreviations are to be used.

It is extremely important to document any data regarding participant's needs. Follow up documentation is to be included as appropriate. If a need is identified in the narrative, any subsequent actions taken on the participant's behalf are to be included in the narrative. All contacts made with the participant or on the participant's behalf must be narrated. Documentation must be kept current. All

contacts and actions must be narrated within 3 business days of the activity, unless a shorter period of time is otherwise specified in policy and procedures, contracts, or other official SCDHHS communications. Information entered in Phoenix Tablet Mode must be uploaded to the network daily.

The date noted in the “Date of Contact” field in the narrative must be the date the activity or contact was completed. The date of the contact may also be entered in the text of the narrative but this is not required since the “Date of Contact” field denotes the date that the contact was made. The date the documentation is entered will automatically be recorded by Phoenix. **Narrative documentation indicating activities completed over several days are not appropriate. Activities completed on a specific date should be documented indicating the appropriate date of contact.** For example, if case activity was completed on July 6th and July 7th and being narrated on July 7th, the worker will enter one narrative with a date of contact of July 6th and a separate narrative with a date of contact of July 7th.

All documentation for the month must be keyed and uploaded to the network by 11:59 pm of the last day of the month. This supersedes any documentation timeliness and policy guidelines. For example, documentation related to re-evaluations completed the last week of month but not due until the first week of the following month, must be completed by 11:59 of the month the visit was completed.

When counting days for timeliness standard issues, the first day is determined to be the next business or calendar day depending on the standard. For example, the timeliness standard for documentation is 3 business days. For an activity that occurred on a business day Monday, Day 1 would be Tuesday, Day 2 would be Wednesday and Day 3 would be Thursday. Therefore the activity must be documented by 11:59 pm on Day 3, Thursday. Or for an activity that occurred on a business Day Friday, Day 1 would be Monday, Day 2 would be Tuesday, and Day 3 would be Wednesday. Therefore the activity must be documented by 11:59 pm on Day 3, Wednesday. However, all documentation for the month must be keyed and uploaded to the network by 11:59 PM of the last day of the month regardless of timeliness standards.

The first and last name and title, if applicable, of any person from whom information is received must be documented in the narrative. The primary contact and the physician are to be identified by name in the narrative, as the individuals assuming these roles may change over time.

A fax copy of a document may be accepted and suffice for documentation in the record if legible. If a document is faxed to someone, there is no need to follow-up with an original by mail. The original must be scanned into Phoenix Scans. The overall rule is routing methods, such as mail or fax, should only be used when using Phoenix is not an option or when a back-up copy is also needed.

All documents available in Phoenix are to be completed and maintained in Phoenix.

All documents that are not available in Phoenix are to be maintained in Phoenix by scanning the document to the participant's Phoenix file.

All scanned documents are to be stored under the appropriate scan tag in Phoenix Scans. Please refer to the "Scan Tag Descriptions" document in Phoenix HELP to determine the appropriate scan tag for document placement.

All documents received from outside sources, such as Physician's Information Form and primary case manager packets, etc., are to be scanned into the database by 11:59 PM on the last day of the month unless the document is received within the last 3 business days of the month. In this situation, the document should be scanned into Phoenix by the 5th of the following month.

The hard copy form may be disposed once the documents are confirmed to be present and readable in Phoenix Scans. All participant records must be disposed of following HIPAA policies.

When paper forms are necessary, black ink should be used.

All telephone contacts to participants, primary contacts, providers, etc. must be through verbal information exchange and not by electronic means (i.e. texting, Emailing, tweeting, twexting, etc.)

01.24 Medical Necessity Criteria for the Children's Personal Care Service

An individual qualifies under the Medical Necessity Criteria for Children's Personal Care (CPC) Service if he/she meets the Service Needs Requirement **and** has one of the Functional Deficits listed below.

Service Needs Requirement*

The individual requires daily monitoring and observation due to medical needs which could result in medical complications. The medical needs must be documented, and the services of a personal care aide must be required and intended to maintain

optimum health status.

Functional Deficits **

1. Requires extensive (hands on) assistance with bathing **and** dressing **and** toileting **and** feeding if otherwise age appropriate functioning would normally allow these activities. (All four must be present and constitute one deficit). **Age 3 or above**
2. Requires extensive (hands on) assistance with walking or wheelchair locomotion if these are otherwise age appropriate activities. **Age 1 or above**
3. Requires extensive (hands on) assistance with transfer if otherwise age appropriate activity. **Age 1 or above**
4. Requires extensive (hands on) assistance with daily incontinence care (if continence is otherwise age appropriate) or with daily catheter or ostomy care. **Child should achieve daytime continence by age 3.**

* If the individual meets the Service Needs Requirement, but no Functional Deficit exists, the case must be referred to the CLTC Central office for special review and determination of medical necessity.

** For children 0-5 years of age, use the Level of Care Users Guide's Developmental Guide to determine age-appropriate activity.